



Patient Information	Specimen Information	Client Information
BRADSHAW, BRETT DOB: 03/24/1977 AGE: 47 Gender: M Fasting: Y Phone: NG Patient ID: 105960	Specimen: SA303809Q Requisition: 6385707 Lab Ref #: 10025098 Collected: 05/16/2024 / 11:15 PDT Received: 05/17/2024 / 04:45 PDT Reported: 05/24/2024 / 14:46 PDT	Client #: 78301949 MAIL992 MALCOLMSON, MICHELLE HUMAN LONGEVITY-SF-APAL 600 GATEWAY BLVD STE 101 SOUTH SAN FRANCISCO, CA 94080-7014

COMMENTS: FASTING

Test Name	In Range	Out Of Range	Reference Range	Lab
TESTOSTERONE, FREE (DIALYSIS) AND TOTAL (MS)				
TESTOSTERONE, TOTAL, MS	614		250-1100 ng/dL	EZ
For additional information, please refer to http://education.questdiagnostics.com/faq/TotalTestosteroneL CMSMS (This link is being provided for informational/educational purposes only.)				
TESTOSTERONE, FREE (DIALYSIS)				EZ
TESTOSTERONE, FREE	109.9		35.0-155.0 pg/mL	
This test was developed and its analytical performance characteristics have been determined by Quest Diagnostics. It has not been cleared or approved by FDA. This assay has been validated pursuant to the CLIA regulations and is used for clinical purposes.				
LIPOPROTEIN (a)	<10		nmol/L Reference Range <75	EN
Risk: Optimal <75 Moderate 75-125 High >125 Cardiovascular event risk category cut points (optimal, moderate, high) are based on Tsimika S. JACC 2017;69:692-711.				
HS CRP	0.2		mg/L	UL
Reference Range Optimal <1.0 Jellinger PS et al. Endocr Pract.2017;23(Suppl 2):1-87. For ages >17 Years: hs-CRP mg/L Risk According to AHA/CDC Guidelines <1.0 Lower relative cardiovascular risk. 1.0-3.0 Average relative cardiovascular risk. 3.1-10.0 Higher relative cardiovascular risk. Consider retesting in 1 to 2 weeks to exclude a benign transient elevation in the baseline CRP value secondary to infection or inflammation. >10.0 Persistent elevation, upon retesting, may be associated with infection and inflammation.				
Pearson TA, Mensah GA, Alexander RW, et al. Markers of inflammation and cardiovascular disease: application to clinical and public health practice: A statement for healthcare professionals from the Centers for Disease Control and Prevention and the American Heart Association. Circulation 2003; 107(3):				



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Test Name	In Range	Out Of Range	Reference Range	Lab
499-511. APOLIPOPROTEIN EVALUATION APOLIPOPROTEIN A1	137		mg/dL Reference Range > or = 115 Risk Category: Optimal > or = 115 High <115 Cardiovascular event risk category cut points (optimal, high) are based on the AMORIS study, Walldius G et al. J Intern Med. 2004;255:188-205.	EN
APOLIPOPROTEIN B	78		mg/dL Reference Range < 90 Risk Category: Optimal <90 Moderate 90-119 High > or = 120 Cardiovascular event risk category cut points (optimal, moderate, high) are based on National Lipid Association recommendations-Jacobson TA et al. J of Clin Lipid. 2015; 9: 129-169 and Jellinger PS et al. Endocr Pract. 2017;23(Suppl 2):1-87.	
APOLIPOPROTEIN B/A1 RATIO	0.57		Reference Range < 0.77 Risk Category: Optimal <0.77 Moderate 0.77-0.95 High >0.95 Cardiovascular event risk category cut points (optimal, moderate, high) are based on the AMORIS study, Walldius G et al. J Intern Med. 2004;255:188-205.	
COMPREHENSIVE METABOLIC PANEL				UL
GLUCOSE	86		65-99 mg/dL Fasting reference interval	
UREA NITROGEN (BUN)	17		7-25 mg/dL	
CREATININE	1.03		0.60-1.29 mg/dL	
EGFR	90		> OR = 60 mL/min/1.73m2	
BUN/CREATININE RATIO	SEE NOTE:		6-22 (calc) Not Reported: BUN and Creatinine are within reference range.	
SODIUM	139		135-146 mmol/L	



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Test Name	In Range	Out Of Range	Reference Range	Lab
POTASSIUM	4.0		3.5-5.3 mmol/L	
CHLORIDE	107		98-110 mmol/L	
CARBON DIOXIDE	25		20-32 mmol/L	
CALCIUM	9.4		8.6-10.3 mg/dL	
PROTEIN, TOTAL	6.5		6.1-8.1 g/dL	
ALBUMIN	4.4		3.6-5.1 g/dL	
GLOBULIN	2.1		1.9-3.7 g/dL (calc)	
ALBUMIN/GLOBULIN RATIO	2.1		1.0-2.5 (calc)	
BILIRUBIN, TOTAL	1.2		0.2-1.2 mg/dL	
ALKALINE PHOSPHATASE	60		36-130 U/L	
AST	25		10-40 U/L	
ALT	30		9-46 U/L	
HEMOGLOBIN A1c	5.2		<5.7 % of total Hgb	UL

For the purpose of screening for the presence of diabetes:

<5.7% Consistent with the absence of diabetes
5.7-6.4% Consistent with increased risk for diabetes (prediabetes)
> or =6.5% Consistent with diabetes

This assay result is consistent with a decreased risk of diabetes.

Currently, no consensus exists regarding use of hemoglobin A1c for diagnosis of diabetes in children.

According to American Diabetes Association (ADA) guidelines, hemoglobin A1c <7.0% represents optimal control in non-pregnant diabetic patients. Different metrics may apply to specific patient populations. Standards of Medical Care in Diabetes(ADA).

This test was performed on the Roche cobas c503 platform. Effective 11/13/23, a change in test platforms from the Abbott Architect to the Roche cobas c503 may have shifted HbA1c results compared to historical results. Based on laboratory validation testing conducted at Quest, the Roche platform relative to the Abbott platform had an average increase in HbA1c value of < or = 0.3%. This difference is within accepted variability established by the National Glycohemoglobin Standardization Program. Note that not all individuals will have had a shift in their results and direct comparisons between historical and current results for testing conducted on different platforms is not recommended.

PROTEIN, TOTAL AND PROTEIN ELECTROPHORESIS W/REFL IFE				
PROTEIN, TOTAL, SERUM				EZ
PROTEIN, TOTAL	6.3		6.1-8.1 g/dL	
PROTEIN ELECTROPHORESIS, SERUM				EZ
ALBUMIN	4.1		3.8-4.8 g/dL	
ALPHA 1 GLOBULIN	0.2		0.2-0.3 g/dL	



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Test Name	In Range	Out Of Range	Reference Range	Lab
ALPHA 2 GLOBULIN		0.4 L	0.5-0.9 g/dL	
BETA 1 GLOBULIN	0.4		0.4-0.6 g/dL	
BETA 2 GLOBULIN	0.3		0.2-0.5 g/dL	
GAMMA GLOBULIN	0.8		0.8-1.7 g/dL	
INTERPRETATION	SEE NOTE			
Evaluation reveals an isolated decrease in alpha-2 globulins. This pattern may be the result of decreased haptoglobin due to hemolytic process or hereditary deficiency.				
IMMUNOGLOBULIN A	176		47-310 mg/dL	UL
LIPASE	34		7-60 U/L	UL
TSH	1.38		0.40-4.50 mIU/L	UL
T4, FREE	0.9		0.8-1.8 ng/dL	UL
T3, FREE	3.5		2.3-4.2 pg/mL	UL
THYROGLOBULIN ANTIBODIES	<1		< or = 1 IU/mL	EN
THYROID PEROXIDASE ANTIBODIES	<1		<9 IU/mL	EN
TISSUE TRANSGLUTAMINASE AB, IGA	<1.0		U/mL	EZ

Units Value	Interpretation	
<15.0	Antibody not detected	
> or = 15.0	Antibody detected	
CBC (INCLUDES DIFF/PLT)		UL
WHITE BLOOD CELL COUNT	4.8	3.8-10.8 Thousand/uL
RED BLOOD CELL COUNT	4.71	4.20-5.80 Million/uL
HEMOGLOBIN	14.7	13.2-17.1 g/dL
HEMATOCRIT	42.3	38.5-50.0 %
MCV	89.8	80.0-100.0 fL
MCH	31.2	27.0-33.0 pg
MCHC	34.8	32.0-36.0 g/dL
RDW	14.6	11.0-15.0 %
PLATELET COUNT	247	140-400 Thousand/uL
MPV	11.1	7.5-12.5 fL
ABSOLUTE NEUTROPHILS	2842	1500-7800 cells/uL
ABSOLUTE LYMPHOCYTES	1526	850-3900 cells/uL
ABSOLUTE MONOCYTES	326	200-950 cells/uL
ABSOLUTE EOSINOPHILS	58	15-500 cells/uL
ABSOLUTE BASOPHILS	48	0-200 cells/uL
NEUTROPHILS	59.2	%
LYMPHOCYTES	31.8	%
MONOCYTES	6.8	%
EOSINOPHILS	1.2	%
BASOPHILS	1.0	%
URINALYSIS, COMPLETE		UL
W/REFLEX TO CULTURE		
COLOR	YELLOW	YELLOW
APPEARANCE	CLEAR	CLEAR
SPECIFIC GRAVITY	1.009	1.001-1.035
PH	6.0	5.0-8.0
GLUCOSE	NEGATIVE	NEGATIVE
BILIRUBIN	NEGATIVE	NEGATIVE
KETONES	NEGATIVE	NEGATIVE
OCCULT BLOOD	NEGATIVE	NEGATIVE



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Test Name	In Range	Out Of Range	Reference Range	Lab
PROTEIN	NEGATIVE		NEGATIVE	
NITRITE	NEGATIVE		NEGATIVE	
LEUKOCYTE ESTERASE	NEGATIVE		NEGATIVE	
WBC	NONE SEEN		< OR = 5 /HPF	
RBC	NONE SEEN		< OR = 2 /HPF	
SQUAMOUS EPITHELIAL CELLS	NONE SEEN		< OR = 5 /HPF	
BACTERIA	NONE SEEN		NONE SEEN /HPF	
HYALINE CAST	NONE SEEN		NONE SEEN /LPF	

This urine was analyzed for the presence of WBC, RBC, bacteria, casts, and other formed elements. Only those elements seen were reported.

REFLEXIVE URINE CULTURE				UL
	NO CULTURE INDICATED			
IRON AND TOTAL IRON				UL
BINDING CAPACITY				
IRON, TOTAL	79		50-180 mcg/dL	
IRON BINDING CAPACITY	386		250-425 mcg/dL (calc)	
% SATURATION	20		20-48 % (calc)	
FERRITIN	40		38-380 ng/mL	UL
VITAMIN B12	376		200-1100 pg/mL	UL

Please Note: Although the reference range for vitamin B12 is 200-1100 pg/mL, it has been reported that between 5 and 10% of patients with values between 200 and 400 pg/mL may experience neuropsychiatric and hematologic abnormalities due to occult B12 deficiency; less than 1% of patients with values above 400 pg/mL will have symptoms.

HEPATITIS B CORE ANTIBODY (IGM)	NON-REACTIVE	NON-REACTIVE		UL
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For additional information, please refer to <http://education.questdiagnostics.com/faq/FAQ202> (This link is being provided for informational/educational purposes only.)

DHEA SULFATE	112		61-442 mcg/dL	EN
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DHEA-S values fall with advancing age. For reference, the reference intervals for 31-40 year old patients are:

Male: 93-415 mcg/dL
Female: 19-237 mcg/dL

HEPATITIS A IGM	NON-REACTIVE	NON-REACTIVE		UL
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For additional information, please refer to <http://education.questdiagnostics.com/faq/FAQ202> (This link is being provided for informational/educational purposes only.)



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Test Name	Result	Reference Range	Lab
VITAMIN D,25-OH,TOTAL,IA	83	30-100 ng/mL	UL
Vitamin D Status 25-OH Vitamin D: Deficiency: <20 ng/mL Insufficiency: 20 - 29 ng/mL Optimal: > or = 30 ng/mL For 25-OH Vitamin D testing on patients on D2-supplementation and patients for whom quantitation of D2 and D3 fractions is required, the QuestAssureD(TM) 25-OH VIT D, (D2,D3), LC/MS/MS is recommended: order code 92888 (patients >2yrs). For additional information, please refer to http://education.QuestDiagnostics.com/faq/FAQ199 (This link is being provided for informational/ educational purposes only.) Physician Comments:			

Infectious Diseases

Test Name	Result	Reference Range	Lab
HEPATITIS C AB W/REFL TO HCV RNA, QN, PCR			UL
HEPATITIS C ANTIBODY	NON-REACTIVE	NON-REACTIVE	
HCV antibody was non-reactive. There is no laboratory evidence of HCV infection. In most cases, no further action is required. However, if recent HCV exposure is suspected, a test for HCV RNA (test code 35645) is suggested. For additional information please refer to http://education.questdiagnostics.com/faq/FAQ22v1 (This link is being provided for informational/ educational purposes only.)			
HEPATITIS B SURFACE ANTIGEN W/REFL CONFIRM			UL
HEPATITIS B SURFACE ANTIGEN	NON-REACTIVE	NON-REACTIVE	
For additional information, please refer to http://education.questdiagnostics.com/faq/FAQ202 (This link is being provided for informational/ educational purposes only.) Physician Comments:			

PERFORMING SITE:

EN QUEST DIAGNOSTICS-WEST HILLS, 8401 FALLBROOK AVENUE, WEST HILLS, CA 91304-3226 Laboratory Director: TAB TOOCHINDA,MD, CLIA: 05D0642827
EZ QUEST DIAGNOSTICS/NICHOLS SJ, 33608 ORTEGA HWY, SAN JUAN CAPISTRANO, CA 92675-2042 Laboratory Director: IRINA MARAMICA,MD,PHD,MBA, CLIA: 05D0643352
UL QUEST DIAGNOSTICS SACRAMENTO, 3714 NORTHGATE BLVD, SACRAMENTO, CA 95834-1617 Laboratory Director: M. ROSE AKIN, M.D., FCAP, CLIA: 05D0644209



Patient Information	Specimen Information	Client Information
BRADSHAW, BRETT DOB: 03/24/1977 AGE: 47 Gender: M Phone: NG Patient ID: 105960	Specimen: SA302352Q Requisition: 6381343 Lab Ref #: 10025099 Collected: 05/16/2024 / 23:15 PDT Received: 05/17/2024 / 03:43 PDT Reported: 05/20/2024 / 21:52 PDT	Client #: 78301949 MAIL992 MALCOLMSON, MICHELLE HUMAN LONGEVITY-SF-APAL 600 GATEWAY BLVD STE 101 SOUTH SAN FRANCISCO, CA 94080-7014

Test Name	In Range	Out Of Range	Reference Range	Lab
VITAMIN E (TOCOPHEROL)				Z3E
VITAMIN E, ALPHA TOCOPHEROL	8.7		5.7-19.9 mg/L	
Levels of alpha-tocopherol <5 mg/L are consistent with Vitamin E deficiency in adults.				
VITAMIN E, BETA GAMMA TOCOPHEROL	<1.0		<4.4 mg/L	
(Note)				

Vitamin supplementation within 24 hours prior to blood draw may affect the accuracy of results.

This test was developed and its analytical performance characteristics have been determined by Quest Diagnostics. It has not been cleared or approved by the FDA. This assay has been validated pursuant to the CLIA regulations and is used for clinical purposes.

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Ithiel James L. Frame, MD, PhD



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Cardio IQ®

Test Name	Current		Risk/Reference Interval			Units	Historical
	Result & Risk		Optimal	Moderate	High		
	Optimal	Non-Optimal					Result & Risk
LIPID PANEL W/ TRIGLYCERIDES/HDL-C							
CHOLESTEROL, TOTAL	156		<200	N/A	>=200	mg/dL	
HDL CHOLESTEROL	54		>=40	N/A	<40	mg/dL	
TRIGLYCERIDES	85		<150	150-199	>=200	mg/dL	
LDL CHOLESTEROL	84		<100	100-129	>129	mg/dL (calc)	
CHOL/HDL C	2.9		<=3.5	3.6-5.0	>5.0	calc	
NON HDL CHOLESTEROL	102		<130	130-189	>=190	mg/dL (calc)	
TG/HDL C	1.6		<2.0	2.0-3.0	>3.0	calc	
LIPOPROTEIN FRACTIONATION NMR							
LDL P	1002		<935	935-1816	>1816	nmol/L	
SMALL LDL P	371		<467	467-820	>820	nmol/L	
LDL SIZE	20.9		>20.5	NA	<20.6	nm	
HDL P	32.2		>32.8	29.2-32.8	<29.2	umol/L	
LARGE HDL P	6.7		>7.2	5.3-7.2	<5.3	umol/L	
HDL SIZE	9.1		>9.0	8.7-9.0	<8.7	nm	
LARGE VLDL P	3.0		<3.7	3.7-6.1	>6.1	nmol/L	
VLDL SIZE	48.5		<47.1	47.1-49.0	>49.0	nm	
METABOLIC MARKERS							
INSULIN, INTACT, LC/MS/MS	5		<=16	N/A	>16	uIU/mL	
C-PEPTIDE, LC/MS/MS	1.33		<=2.16	N/A	>2.16	ng/mL	
INSULIN RESISTANCE SCORE	12		<33	33-66	>66		



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For details on reference ranges please refer to the reference range/comment section of the report.

Medical Information For Healthcare Providers: If you have questions about any of the tests in our Cardio IQ offering, please call Client Services at our Quest Diagnostics-Cleveland HeartLab Cardiometabolic Center of Excellence. They can be reached at 866.358.9828, option 1 to arrange a consult with our clinical education team.



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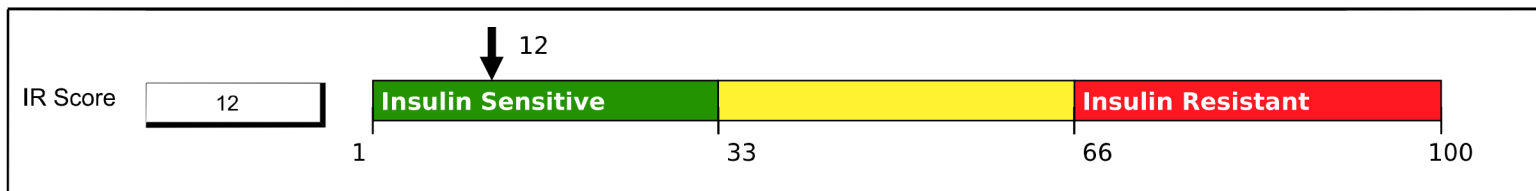
Cardio IQ® Insulin Resistance Panel with Score

Panel Results

Lab: EZ

Test name	Patient Results	Reference Range
INSULIN, INTACT, LC/MS/MS	5	< OR = 16 uIU/mL
C-PEPTIDE, LC/MS/MS	1.33	0.68-2.16 ng/mL
INSULIN RESISTANCE SCORE	12	< OR = 66

Insulin Resistance Score



A score below 33 is optimal. The insulin resistance score correlates with steady state glucose levels achieved during an insulin suppression test, a standard research test for insulin resistance. The score is based on insulin and C-peptide results (Abbasi, F., Shiffman, D., Tong, C.H., Devlin, J. J., Reaven, G. M., McPhaul, M. J. (2017) Identification of Insulin Resistance in Apparently Healthy Individuals. Manuscript in preparation).

Insulin Sensitive < 33; Impaired Insulin Sensitivity 33-66; Insulin Resistant >66

A score below 33 is optimal. The insulin resistance score correlates with steady state glucose levels achieved during an insulin suppression test, a standard research test for insulin resistance. The score is based on insulin and C-peptide results (Abbasi F, Shiffman D, Tong CH, et al. Insulin resistance probability scores for apparently healthy individuals. J Endocr Soc. 2018;2(9):1050-1057).

For additional information, please refer to <http://education.QuestDiagnostics.com/faq/FAQ205> (This link is being provided for informational/educational purposes only.)

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Reference Range/Comments

Analyte Name	In Range	Out Range	Reference Range	Lab
HDL P		32.2	>32.8 umol/L	Z4M
Relative risk: Optimal >32.8; Moderate 29.2-32.8; High <29.2 umol/L. Reference range is 21.1-43.4 umol/L.				
LARGE HDL P		6.7	>7.2 umol/L	Z4M
Relative risk: Optimal >7.2; Moderate 5.3-7.2; High <5.3 umol/L. Reference range is >3.5 umol/L.				
LDL P		1002	<935 nmol/L	Z4M
This test was developed and its analytical performance characteristics have been determined by Quest Diagnostics Cardiometabolic Center of Excellence at Cleveland HeartLab. It has not been cleared or approved by the U.S. Food and Drug Administration. This assay has been validated pursuant to the CLIA regulations and is used for clinical purposes. Relative risk: Optimal <935; Moderate 935-1816; High >1816 nmol/L. Reference range is 592-2404 nmol/L.				
VLDL SIZE		48.5	<47.1 nm	Z4M
Relative risk: Optimal <47.1; Moderate 47.1-49.0; High >49.0 nm. Reference range is 41.1-61.7 nm.				
C-PEPTIDE, LC/MS/MS	1.33		0.68-2.16 ng/mL	EZ
CHOL/HDL C	2.9		<5.0 calc	Z4M
CHOLESTEROL, TOTAL	156		<200 mg/dL	Z4M
HDL CHOLESTEROL	54		>39 mg/dL	Z4M
HDL SIZE	9.1		>9.0 nm	Z4M
Relative risk: Optimal >9.0; Moderate 8.7-9.0; High <8.7 nm. Reference range is 8.3-10.5 nm.				
INSULIN RESISTANCE SCORE	12		< OR = 66	EZ
Insulin Sensitive < 33; Impaired Insulin Sensitivity 33-66; Insulin Resistant >66 A score below 33 is optimal. The insulin resistance score correlates with steady state glucose levels achieved during an insulin suppression test, a standard research test for insulin resistance. The score is based on insulin and C-peptide results (Abbasi F, Shiffman D, Tong CH, et al. Insulin resistance probability scores for apparently healthy individuals. J Endocr Soc. 2018;2(9):1050-1057). For additional information, please refer to http://education.QuestDiagnostics.com/faq/FAQ205 (This link is being provided for informational/educational purposes only.) This test was developed and its analytical performance characteristics have been determined by Quest Diagnostics. It has not been cleared or approved by FDA. This assay has been validated pursuant to the CLIA regulations and is used for clinical purposes.				
INSULIN, INTACT, LC/MS/MS	5		< OR = 16 uIU/mL	EZ
Insulin concentration can be converted to pmol/L by applying the conversion factor: 1 uIU/mL = 5.97 pmol/L For additional information, please refer to http://education.QuestDiagnostics.com/faq/FAQ170 (This link is being provided for informational/educational purposes only.) This test was developed and its analytical performance characteristics have been determined by Quest Diagnostics. It has not been cleared or approved by FDA. This assay has been validated pursuant to the CLIA regulations and is used for clinical purposes.				
LARGE VLDL P	3.0		<3.7 nmol/L	Z4M
Relative risk: Optimal <3.7; Moderate 3.7-6.1; High >6.1 nmol/L. Reference range is <16.0 nmol/L.				
LDL CHOLESTEROL	84		<100 mg/dL (calc)	Z4M
Desirable range <100 mg/dL for primary prevention; <70 mg/dL for patients with CHD or diabetic patients with >= 2 CHD risk factors. LDL-C is now calculated using the Martin-Hopkins calculation, which is a validated novel method providing better accuracy than the Friedewald equation in the estimation of LDL-C. Martin SS et al. JAMA. 2013;310(19): 2061-2068 (http://education.QuestDiagnostics.com/faq/FAQ164)				
LDL SIZE	20.9		>20.5 nm	Z4M
Relative risk: Optimal >20.5; High <20.6 nm. Reference range is 20.0-22.3 nm.				
NON HDL CHOLESTEROL	102		<130 mg/dL (calc)	Z4M
For patients with diabetes plus 1 major ASCVD risk factor, treating to a non-HDL-C goal of <100 mg/dL (LDL-C of <70 mg/dL) is considered a therapeutic option.				
SMALL LDL P	371		<467 nmol/L	Z4M
Relative risk: Optimal <467; Moderate 467-820; High >820 nmol/L. Reference range is <1408 nmol/L.				
TG/HDL C	1.6		<2.0 calc	Z4M
TRIGLYCERIDES	85		<150 mg/dL	Z4M



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PERFORMING SITE:

EZ QUEST DIAGNOSTICS/NICHOLS SJC, 33608 ORTEGA HWY, SAN JUAN CAPISTRANO, CA 92675-2042 Laboratory Director: IRINA MARAMICA,MD,PHD,MBA, CLIA: 05D0643352
Z3E MEDFUSION, 2501 SOUTH STATE HIGHWAY 121 SUITE 1100, LEWISVILLE, TX 75067-8188 Laboratory Director: ITHIEL J FRAME,MD,PHD, CLIA: 45D2004217
Z4M CLEVELAND HEARTLAB INC, 6701 CARNEGIE AVENUE SUITE 500, CLEVELAND, OH 44103-4623 Laboratory Director: SAMI ALBEIROTI,PHD,DABCC, CLIA: 36D1032987

BRADSHAW, BRETT

DOB: 03/24/1977 Age: 47 Specimen: SA296748Q Collected: 05/16/2024 11:15 Client #: 78301949
Sex: M Fasting: Y Requisition: 6349682 Received: 05/16/2024 23:38 MALCOLMSON, MICHELLE
Phone: Lab Reference ID: 10025100 Reported: 05/20/2024 15:07 HUMAN LONGEVITY-SF-APAL
Patient ID: 105960 Report Status: FINAL 600 GATEWAY BLVD STE 101
SOUTH SAN FRANC, CA 94080-7014
Phone: (844) 838-3322
Fax: (858) 430-2741

FASTING

PSA, TOTAL WITH REFLEX TO PSA, FREE

FINAL

Lab: EN

Analyte	Value
PSA, TOTAL (2857-1)	1.0 Reference Range: < OR = 4.0 ng/mL FINAL
The Total PSA value from this assay system is standardized against the equimolar PSA standard. The test result will be approximately 20% higher when compared to the WHO-standardized Total PSA (Siemens assay). Comparison of serial PSA results should be interpreted with this fact in mind. PSA was performed using the Beckman Coulter Immunoassay method. Values obtained from different assay methods cannot be used interchangeably. PSA levels, regardless of value, should not be interpreted as absolute evidence of the presence or absence of disease.	

Performing Sites

EN Quest Diagnostics-West Hills, 8401 Fallbrook Ave, West Hills, CA 91304-3226 Laboratory Director: Thomas J McDonald

Key

 Priority Out of Range  Out of Range **PEND** Pending Result **PRE** Preliminary Result **FINAL** Final Result **RE** Reissued Result

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